RWANDA NATIONAL ASSOCIATION OF DEAF WOMEN **(RNADW "UMUCYO")**

BEYOND SILENCE

HEALTH SECTOR POLICY BRIEF

A POLICY BRIEF ON INTEGRATING RWANDAN SIGN LANGUAGE INTO RWANDA'S HEALTH SECTOR

KIGALI - RWANDA

SEPTEMBER 2025

A POLICY BRIEF ON INTEGRATING RWANDAN SIGN LANGUAGE INTO RWANDA'S HEALTH SECTOR

CONTENTS

1. Executive Summary 02

2. Introduction: The Right to Health and the Reality of Exclusion 03

3. The Depth of the Systemic Barrier: A Granular Analysis 04

4. The Tangible and Intangible Costs of Exclusion 05

5. The Solution: A Detailed Blueprint for Integration 06

6. The Compelling Return on Investment (ROI) 07

7. Conclusion and Specific, Actionable Recommendations 08

1. EXECUTIVE SUMMARY

Deaf citizens, particularly adolescent girls and young women (AGYW), face a systemic barrier to healthcare due to the absence of Rwandan Sign Language (RSL) services, leading to dire health outcomes and violating their fundamental rights.

This brief quantifies the long-term financial and public health burden of misdiagnosis, late-stage disease presentation, and non-compliance with the Convention on the Rights of Persons with Disabilities (CRPD).

The solution lies in a strategic investment in a national programme for RSL medical interpreters and mandatory provider training. This is not merely an expense but an investment that will yield improved health outcomes, greater system efficiency, CRPD compliance, and significant economic gains.

We recommend the immediate establishment of a dedicated budget line and a National RSL in the Health Unit within the Rwanda Biomedical Centre (RBC) to coordinate and sustain these efforts.

We further recommend piloting this programme in Nyarugenge and Huye districts within the next fiscal year to demonstrate its efficacy and refine the national scale-up model.

2. INTRODUCTION: THE RIGHT TO HEALTH AND THE REALITY OF EXCLUSION

Rwanda's commitment to Universal Health Coverage and the CRPD stands in stark contrast to the lived experience of Deaf patients.

Despite progressive policies, systemic barriers within the health sector prevent Deaf individuals from accessing quality care.

The lack of RSL services directly undermines NST2's Social Transformation pillar, which commits to strengthened health systems and inclusive development for Persons with Disabilities (PwD), thereby hindering progress toward Universal Health Coverage (UHC) and the principle of leaving no one behind.

This brief focuses on these barriers and the economic imperative to remove them.

Our methodology synthesises data from rapid assessments, National Council of Persons with Disabilities (NCPD) reports[1], and global evidence[2] on the cost of health exclusion, providing a comprehensive analysis of the current landscape and an actionable path forward.

1 NCPD Report on Health Access (2023).
2 RNADW Community Needs Assessment (2025);

3. THE DEPTH OF THE SYSTEMIC BARRIER: A GRANULAR ANALYSIS

3.1. The Communication Chasm

Fewer than **5%** of healthcare facilities in Rwanda have staff proficient in RSL, resulting in a significant communication gap.

Deaf patients often rely on children or untrained family members as interpreters, leading to breaches of confidentiality and inaccurate information exchange.

Detailed patient journeys reveal that note-writing and gestures are inadequate substitutes for fluent communication, violating informed consent and patient safety protocols.

This chasm not only impedes care but also erodes trust between Deaf patients and healthcare providers.

3.2. The Knowledge Gap Among Providers

Healthcare workers frequently lack training on Deaf culture and audism, leading to ableist assumptions that Deaf patients are “too difficult” to serve.

This results in skipped consultations, particularly for SRHR services, where sensitive topics require clear communication.

A case study of SRHR counselling reveals that Deaf clients are often overlooked or rushed through appointments, exacerbating their vulnerability to preventable health issues.

3.3. Inaccessible Infrastructure and Materials

Health facilities lack visual aids, RSL video screens in waiting rooms, and accessible appointment systems.

Even youth-friendly YEGO corners, designed to serve vulnerable adolescents, fail to accommodate Deaf youth.

This institutional neglect reinforces exclusion and limits the effectiveness of otherwise well-designed health initiatives.

4. THE TANGIBLE AND INTANGIBLE COSTS OF EXCLUSION

4.1. Public Health Costs

The financial burden of misdiagnosis and communication errors encompasses costs associated with incorrect prescriptions, repeated visits, and unnecessary tests.

Late presentation of diseases: For example, treating late-stage cervical cancer is exponentially more costly than preventive screening and early intervention, generating significant savings for the CBHI scheme.

These avoidable costs directly undermine NST2's Social Transformation priorities. They hinder the goals of reducing disease mortality through improved maternal and child health and compromise the achievement of UHC, which depends on equitable access through schemes like the Community-Based Health Insurance (CBHI) for PwD.

Additionally, the economic impact of avoided care, where Deaf young people forgo preventive services like vaccinations, family planning, and STI testing due to prior negative experiences, further exacerbates public health challenges and increases long-term costs.

4.2. Socio-Economic Costs

Deaf individuals often cannot work due to preventable illnesses, reducing productivity and economic participation.

The intergenerational cost of unplanned pregnancies and poor maternal health perpetuates cycles of poverty and inequality, affecting not only individuals but also families and communities.

4.3. Legal and Reputational Costs

Non-compliance with the CRPD exposes Rwanda to potential litigation and undermines its reputation as a leader in health and development.

The gap between policy and practice risks damaging international partnerships and donor confidence.

5. THE SOLUTION: A DETAILED BLUEPRINT FOR INTEGRATION

5.1. Pillar 1: Building a National Corps of RSL Medical Interpreters

* **Recruit & Train:** Recruit candidates from the Deaf community and hearing allies. Train them with a standardized curriculum in medical terminology, ethics, and confidentiality.
* **Deploy & Manage:** Establish a mixed employment model, including full-time staff at national & referral hospitals, and a pooled, on-call system for district health centres.
* **Ensure Quality:** Implement a national certification and licensing system to guarantee high-quality interpretation services.

5.2. Pillar 2: Mandatory Disability Inclusion & RSL Training for Health Workers

* Co-designed with Deaf stakeholders, this training will cover Deaf culture, audism, and basic RSL health phrases.
* It will be integrated into pre-service education for doctors and nurses and mandated as part of continuous professional development (CPD) for existing staff.
* Accreditation will be linked to professional licensing to ensure compliance with relevant regulations.

5.3. Pillar 3: Developing Accessible Health Information

* **Create Digital Library:** Produce a public library of RSL video resources on key SRHR and general health topics.
* **Develop Physical Tools:** Roll out pictogram-based health forms and visual communication boards for triage and consultations in every facility.
* **Ensure Access:** Host videos on a dedicated MoH/RBC online portal and install screens in waiting areas of youth-friendly corners and health centres.

This includes producing a library of RSL video resources on key SRHR topics and developing pictogram-based health forms and communication boards for triage. These materials will ensure that Deaf patients can access information independently and confidently.

6. THE COMPELLING RETURN ON INVESTMENT (ROI)

6.1. Health ROI

Investing in RSL integration will reduce the disease burden, maternal mortality, and STI transmission through earlier diagnosis and better adherence to treatment plans.

These gains directly advance NST2's Social Transformation targets for reduced mortality and strengthened, resilient health systems, delivering measurable progress on key national public health indicators, and are in line with the World Health Organisation's Universal Health Coverage and "health care for all.

6.2. Economic ROI

Savings will accrue from reduced hospitalisations, fewer repeated consultations, and a more productive workforce.

The upfront costs of training and employment will be offset by long-term efficiencies.

6.3. Social ROI

Achieving CRPD compliance will enhance Rwanda's human rights indicators and empower a marginalised community, fostering inclusivity and social cohesion.

Achieving CRPD compliance will enhance Rwanda's human rights record and empower a marginalised community.

This empowerment aligns with NST2's focus on inclusive development, fostering greater social cohesion and fulfilling the rights of PwD.

7. CONCLUSION AND SPECIFIC, ACTIONABLE RECOMMENDATIONS

- We recommend that the Ministry of Health and RBC establish a National RSL Unit, with a dedicated budget and mandate.
- Additionally, we propose piloting the interpreter programme in two districts (one urban, one rural) within the next fiscal year and issuing a ministerial order mandating disability inclusion and basic RSL training with completion linked to professional licensing. for all health sector employees.
- This investment is essential for the success of the Health Sector Strategic Plan V and Rwanda's commitment to leaving no one behind.
- Success will be measured by a defined set of Key Performance Indicators (KPIs), including the percentage of Deaf patients reporting improved communication.

...we propose piloting the interpreter programme...

Rwanda National Association of Deaf Women
RNADW "UMUCYO"
KVINNA KVINNA
C.J.S.M

URGENT ACTION FUND AFRICA

FOR WOMN'S HUMAN RIGHTS